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CLIENT WEBSITE AUDIT / OCTOBER 16, 2026

Dunamis Place

A website built around the way Dunamis works.

Prepared for Mr. Daniel / Nate Mamo, AIA KINGS / Magnolia, Texas

Rebuild the website around admissions.

A complete custom site, planned around the way families, referrers and the intake team actually work.

Mr. Daniel,

I recommend replacing the current WordPress website with a complete custom build. The public review found an unfinished eligibility page, unclear care-service wording, a CAPTCHA quota warning and inconsistent search signals. Those findings support a broader project: rebuild the path from a family's first question to a confirmed next step.

The site already works over HTTPS and publishes a functioning sitemap. All **23 listed HTML pages** returned successfully. Mobile navigation worked at the tested width, and the inspected homepage and appointment layouts fit without horizontal overflow. Existing WP Statistics code is visible, so analytics should be inspected before assuming they are missing. Its collection quality and usefulness for admissions remain unverified. Current website

My proposal is to rebuild the content, page structure, inquiry experience and measurement together. I would preserve useful URLs, contact routes and approved facility information, then deliver a clear handover with named responsibilities. The discovery call establishes care facts, access and delivery scope; the recommendation is a full custom rebuild.

Complete the care-fit story.

Two verified content priorities need administrator-approved answers.

The homepage directs visitors to determine care eligibility, but the destination contains only a holding message. This is a verified information gap. It does not establish how many visitors leave. Eligibility page

Replace it with administrator-approved suitability guidance, assessment steps, important care limits and a clear admissions contact. Families should understand what can be discussed first and what requires a professional assessment. Clinical criteria must come from the responsible operator.

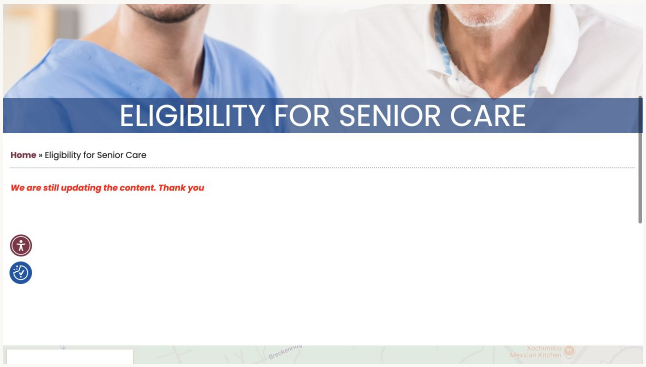
Check the result: approved guidance is published, the contact route works, and staff can compare qualified inquiries and repeated eligibility questions with an agreed baseline.

Clarify who provides each service.

The Adult Day Care page groups home health, hospice, medical equipment, visiting physicians and physical therapy without distinguishing direct provision from outside coordination. The Alzheimer's & Dementia page also reuses adult-day-care language. These are observed wording problems, not findings that services are unavailable or unauthorized. Adult Day Care · Dementia page

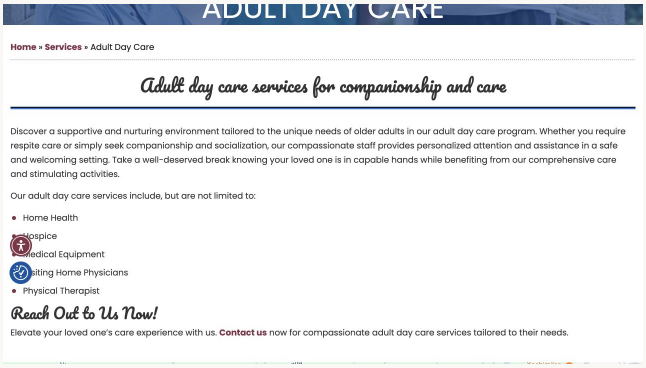
Management should approve a service map that separates residential support, any daytime program, memory-related support and coordinated outside providers. Publish the setting, who provides the service and how suitability is assessed. The official Texas HHSC directory matches Dunamis Place's Magnolia address and Type A assisted-living identity; detailed care claims still need operator approval. HHSC directory

Check the result: each care claim has an approved factual basis, and families/referrers have a clear next step.



October 16, 7:32:48 p.m. CDT. Eligibility contains a holding message.

Source: https://www.dunamisplace.com/assisted-living-home-eligibility-for-senior-care



October 16, 7:32:33 p.m. CDT. Service categories lack provider roles.

Source: https://www.dunamisplace.com/assisted-living-home-services/adult-day-care

Make every inquiry’s next step clear.

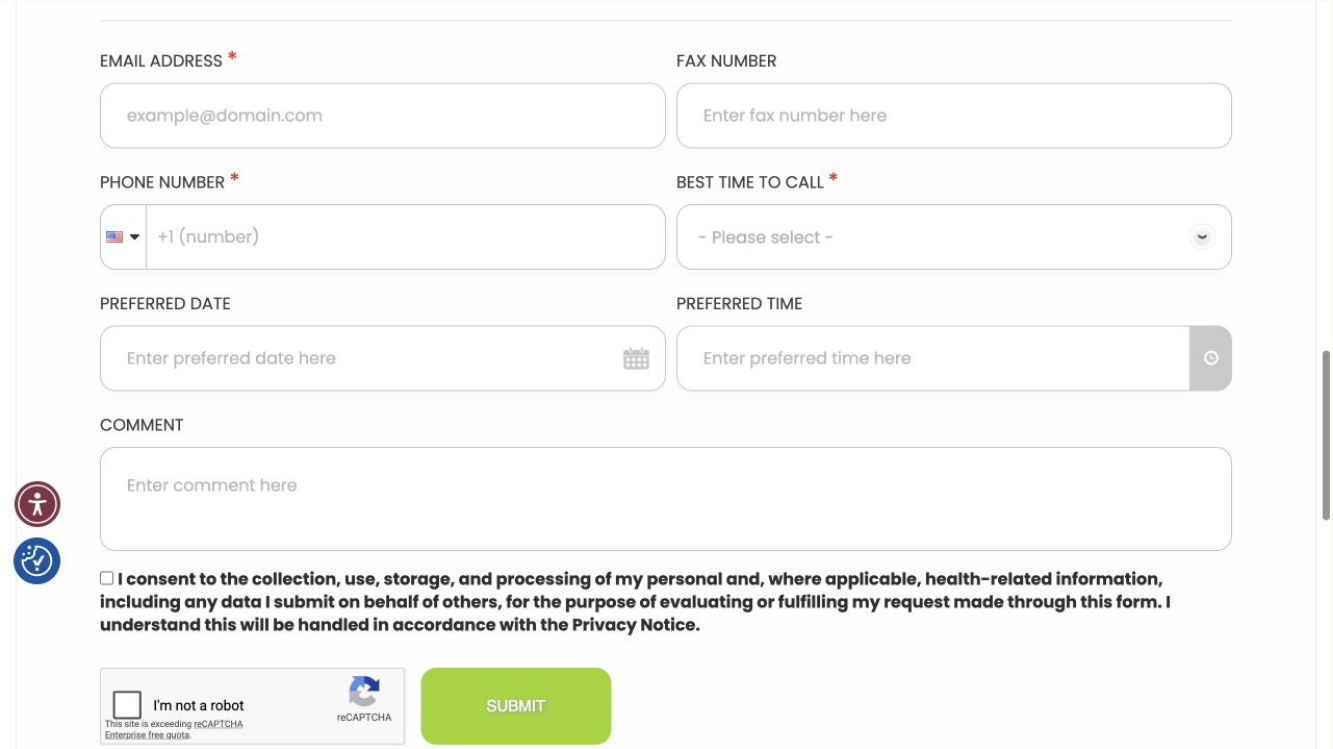
Forms load, but receipt, confirmation and staff follow-up need an authorized test.

A visible reCAPTCHA quota warning appears near inquiry forms. It warrants a configuration review. **This audit did not establish submission failure.** No forms were submitted or fake patient details used. Consent and a Privacy Notice are already present. Appointment page

The appointment form asks for extensive contact/address details and preferred timing, without clearly explaining whether sending it confirms a visit. Review the minimum information needed for an initial callback, then state how a visit is confirmed and who responds.

With authorization, test submission, receipt, acknowledgment and staff ownership using nonpatient test data. Review the privacy notice’s template mismatch and actual information handling with the responsible adviser before expanding tracking or follow-up. Keep care details out of marketing event data. Public inspection cannot determine HIPAA status or certify compliance.

Check the result: an inquiry reaches the assigned person, the family receives accurate next steps and a confirmed tour is distinguishable from a request.



October 16, 7:31:01 p.m. CDT. Consent and a CAPTCHA quota warning are visible. Submission failure was not established.

Source: https://www.dunamisplace.com/assisted-living-home-set-an-appointment

Nine reasons to rebuild Dunamis Place.

One complete replacement, with the improvements tied to concrete checks.

The findings affect the full journey from understanding care to requesting a visit. I recommend rebuilding that journey as one coordinated site, with these nine priorities built into the scope.

1. Finish the admission journey. The linked eligibility page is still a placeholder. I would give it a complete, operator-approved explanation of suitability, assessment and next steps, connected to the right inquiry route.

2. Separate the care pathways. Adult Day Care mixes service categories and the dementia page repeats adult-day-care wording. I would organize approved residential, daytime, memory-related and coordinated services into distinct paths that families and professional referrers can understand.

3. Make the first contact simpler. The appointment form asks for address details and timing without clearly explaining confirmation. I would build a focused callback or tour request, explain what happens next and collect later care information through an approved channel.

4. Make inquiry handling testable. The visible CAPTCHA quota warning needs review; delivery has not been tested. I would include a controlled receipt, acknowledgment and follow-up test, a named staff owner, and a clear distinction between a request and a confirmed tour.

5. Rebuild the search foundation consistently. All 23 inspected pages use HTTP preferred-address tags on HTTPS pages and repeat one generic description; crawler rules block public resources. I would generate consistent HTTPS signals, page-specific descriptions and accessible public resources, then verify the result.

6. Put Magnolia and the people behind the care forward. The address and amenities already provide useful detail, while the main positioning and staff page underuse it. I would build the page hierarchy around the actual location, approved care facts, authentic facility images and permissioned staff credentials.

7. Improve the mobile and accessible care journey. The tested mobile layout works, but a tall header delays the care message. Embedded forms lack descriptive titles, and meaningful gallery images need review. I would bring care and contact forward and test keyboard, screen-reader and mobile tasks.

8. Connect measurement to admissions. WP Statistics code already exists; usable inquiry-to-admission attribution remains unverified. I would review that baseline and design source, staff ownership and stage reporting around inquiries, tours and admission outcomes, with sensitive details kept out of marketing tools.

9. Establish clear ownership and maintenance. Public source shows a WordPress theme, plugins and embedded forms; support responsibilities need confirmation. My proposed replacement removes the WordPress theme/plugin layer and includes documented content access, integration owners, maintenance responsibilities, backup and recovery checks.

A complete custom rebuild, with a clear handover.

This is the delivery approach I recommend for Dunamis Place.

I would replace the WordPress presentation and form layer with a purpose-built site organized around approved care information, a tested inquiry route and documented ownership. The rebuild would bring these parts into one plan, while preserving the current domain and useful search assets. Every part would have an acceptance check before launch.

Current Dunamis build	Proposed improvement	Acceptance check
Eligibility and service-copy gaps	Approved, separate care pathways	Administrator sign-off
Real forms; outcomes unverified	Named owner and clear confirmation	Authorized receipt/follow-up test
HTTPS works; search signals conflict	Consistent HTTPS addresses and crawler access	URL and rendered-page checks
Existing tracking code	Validated source/stage reporting	Reconciled inquiry and tour records

First: approve care wording, reconcile public contact numbers, review CAPTCHA settings and walk through intake. **Next:** scope the custom build with preserved URLs, permanent redirects where needed, accessible forms, measured mobile performance and a rollback plan. **Then:** improve referral/source records and follow-up using real operating evidence.

Future administrative assistance should start with one narrow nonclinical task, minimal access and human approval for consequential actions. Any work involving sensitive information needs appropriate privacy/security review before implementation. Care decisions remain with qualified people.

Plan the rebuild in a focused discovery call.

Confirm the care facts, access and responsibilities needed to deliver the new site.

I suggest walking through the actual admission/referral process, approving the care-service map and reviewing the reporting already available. Confirm who owns the domain/hosting, who receives inquiries and who approves privacy and service language. Those answers determine a realistic scope and timetable.

Measure inquiry received, contacted, qualified, tour scheduled, tour attended and admission outcome using agreed definitions. Track source and response ownership without sensitive detail in marketing tools. There is no verified baseline or projected financial return in this brief.

My recommendation is a full custom rebuild. I want to agree the priorities with you, confirm the care facts and access, and define the migration around how Dunamis handles inquiries and admissions.

Reply to my text to arrange a 15-minute walkthrough.

Discuss the audit or request the detailed findings

Method, evidence and limits

Independent public inspection: October 16, 2026, America/Chicago. Desktop 1280 × 720; responsive 390 × 844; sitemap/HTML and public headers checked. Original screenshots are embedded unchanged. No forms, calls, private records or account changes were made. Backend delivery, current analytics quality and search performance require access. Public PageSpeed testing returned a shared-quota error, so no speed score is reported.

Preferred-address tags and public crawler rules need correction and Search Console checks. Clear content and crawl access support discoverability; no special host, markup or AI file guarantees inclusion in search or AI answers. Google AI guidance · Canonical guidance · Accessible forms · Current Privacy Notice

The full independent audit is available on request from Nate Mamo. Contact: nate@aiautomationkings.com.